

Clinical Practice Note: Management of Stage 1 Hypertension

****Patient encounter — primary care follow-up****

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Subjective

The patient is a 58-year-old presenting for a six-month hypertension follow-up. Since the last visit, the patient reports occasional morning headaches and mild fatigue but no chest pain, palpitations, dyspnea on exertion, peripheral edema, or syncope episodes. Medication adherence has been good. Home blood pressure monitoring records were brought to the visit and reviewed.

The patient has no recent changes to diet, exercise, or sleep. They report moderate alcohol intake (two to three drinks per week), no tobacco use, and a recent attempt to increase walking to thirty minutes most days. Family history remains notable for early cardiovascular disease in a first-degree relative.

Objective

In-clinic measurements taken after five minutes of quiet seated rest, on the non-dominant arm with an appropriately sized cuff. The patient's average systolic blood pressure across three readings was 138 mmHg with a diastolic of 86 mmHg. Heart rate was 72 and regular. BMI was 28.4. The remainder of the focused cardiovascular examination was unremarkable: no carotid bruits, normal S1 and S2 without murmurs or gallops, no jugular venous distention, and no peripheral edema. Recent laboratory values from the patient's home draw kit were within the expected ranges for renal function, electrolytes, and lipids.

Assessment

Stage 1 hypertension, partially controlled on current monotherapy. The patient meets the diagnostic criteria for sustained hypertension based on both in-office and out-of-office measurements, with consistent elevation above the 130/80 threshold across multiple visits. The presence of an early cardiovascular family history places this patient in a higher absolute risk category than the office numbers alone would suggest.

Plan

We discussed treatment intensification and reviewed the rationale for adding a second agent versus increasing the current dose. Given the patient's preference to minimize medication burden, we elected to first attempt to optimize lifestyle factors over the next eight weeks before adding a second pharmacologic agent. The patient agreed to a structured walking program, a DASH-style dietary trial with reduced sodium, and weight reduction targeting a five-percent decrease over three months.

First-line treatment for hypertension at this stage typically includes lifestyle modification combined with ACE inhibitors or angiotensin receptor blockers, both of which reduce vasoconstriction by acting on the renin-angiotensin system. ACE inhibitors block the conversion of angiotensin I to angiotensin II, producing vasodilation, reducing arterial wall tension, and lowering both preload and afterload. Calcium channel blockers and thiazide diuretics remain appropriate alternatives. Beta-blockers are not first-line for uncomplicated hypertension but remain valuable when a concurrent indication exists.

If home blood pressure averages do not fall below 130/80 within eight weeks of lifestyle optimization, we will add a second agent at the next visit. Given this patient's profile, a low-dose thiazide added to the existing ACE inhibitor is the most likely next step. We discussed potential adverse effects including mild cough with ACE inhibitors, electrolyte shifts with thiazides, and the importance of reporting any new symptoms promptly.

Regular monitoring is essential for patients with comorbidities or elevated baseline risk. The patient was instructed to continue daily home blood pressure measurements, recording morning and evening readings, and to bring the log to the next visit. We reviewed proper measurement technique, including cuff size, arm support, and the value of repeated readings after a brief rest. The patient demonstrated good understanding of the plan and was provided with written materials summarizing the diagnosis, the treatment goals, and the warning signs that would warrant earlier follow-up — including persistent headache, visual changes, chest discomfort, or readings consistently above 160/100.

Follow-up appointment scheduled in eight weeks. Telephone outreach by the clinical pharmacist in four weeks to assess adherence and tolerability.